Royal Devon University Healthcare NHS Foundation Trust

Patients Declining a Blood Transfusion Policy	
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Please *specify* standard/criterion numbers and tick ✓ other boxes as appropriate

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Other *(please specify)*:			
Note: This document has been assessed for any equality, diversity or human rights implications			

Controlled document
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1.1	April 2025	CNS Intravascular Fluid Management	Appx 1 Contact details updated for the Hospital Liaison Committee for Jehovah's Witnesses.
1.2	June 2025	CNS Intravascular Fluid Management	Presented for review at Hospital Transfusion Team meeting 19/06/2025. No amendments required.

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Executive Lead Signature: *(Applicable only to Trust Strategies & Policies)*	

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KEY POINTS OF THIS POLICY:

- To provide guidance to staff involved in the care of patients who may decline blood product support.
- To provide assurance to patients who decline blood product support that their wishes will be respected.

1 INTRODUCTION

1.1. This document has been produced to ensure that Royal Devon University Healthcare NHS Foundation Trust (RDUH) supports all patients with cultural, spiritual or personal beliefs which impact on their willingness to receive blood components or products.

1.2. In British law, the fully informed competent adult patient has an absolute right to accept or decline medical treatment. Therefore, to administer blood in the face of an informed declination by the patient may invoke criminal and/or civil proceedings.

1.3. An accurate record of all discussions about consent and treatment options must be made in the patient's record. (RDUH Consent Policy)

1.4. The patient is entitled to change their mind regarding treatment at any time and if the declination of a blood transfusion is revoked (written or verbal) it should be clearly documented in the patient's record.

1.5 If the individual lacks capacity refer to the Trust's Mental Capacity Act Policy.

2 PURPOSE

2.1 The purpose of this document is to ensure adherence to HSC 2001/023: Good practice in consent: Achieving the NHS Plan commitment to patient centred consent practice (2007).

2.2 The applicable UK laws are:

- Mental Health Act 1983 Part III (amended 2007).
- Mental Capacity Act 2005
- Family Law Reform Act 1969.
- The Children Act 2004

2.3 The policy applies to clinical staff involved with patients who require or may require a blood component or product transfusion.

2.4 Implementation of this policy will ensure that the wishes and beliefs of all patients are acknowledged and respected and that information and guidance is provided to clinicians on the acceptable management of these patients.

3 DEFINITIONS

3.1 **Advance Decision**
This is a decision to decline specific medical treatment in advance. This is valid if made voluntarily by an appropriately informed adult with capacity. The Mental Capacity Act (2005) sets out the requirements of an Advance Decision. Please refer to the Trust's Mental Capacity Act Policy.

3.2 **Blood Component**

Blood Components, namely red blood cells, white blood cells, platelets and plasma derived from a single unit of donated blood

3.3 **Blood Products or Fractionated Blood Products**
These are products containing plasma derivatives from more than one blood donation pooled together. They are commonly known as fractionated blood products by Jehovah Witness patients and include human albumin solution, clotting fraction concentrates, immunoglobulin solutions.

3.4 **Allogeneic Blood**
Donated blood

3.5 **Autologous blood**
Patient's own blood collected for re-infusion.

4 DUTIES AND RESPONSIBILITIES OF STAFF

4.1 The **Chief Medical Officer** is responsible for ensuring that all clinical staff are aware of this Trust policy and its implications for patients.

4.2 The **Trust Solicitors** are responsible for application to the courts for any legal "specific issue orders" which may be required in the case of juvenile patients.

4.3 The **Consultant in charge** is responsible for ensuring that the wishes of the patient are respected. In the case of juvenile patients, the consultant is responsible for ensuring that timely applications are made to the Executive On-Call for legal intervention if required. If a person with parental responsibility declines permission for a blood transfusion, it may be necessary for the Consultant to apply for a legal "Specific Issue Order" via the High Court.

4.4 **Laboratory staff** are responsible for informing the clinician if the patient's record on laboratory system has an historic note indicating "no blood products" (prior to EPIC use).

4.5 All staff must ensure the policy is followed whenever a patient declines a blood transfusion as part of their treatment. The member of staff discussing the patient's wishes regarding blood transfusions is responsible for ensuring that all discussions on treatment options are recorded in the patient's records.

5 JEHOVAH'S WITNESSES

5.1 Any competent adult has the right to decline blood. Jehovah's Witnesses are the most well recognised group of individuals who may decline blood component or product support.

5.2 Their decisions are not related to the perceived risk of transfusion but are a deeply held core belief based on biblical scriptures. Furthermore they regard a non-consensual transfusion as a physical violation.

5.3 Jehovah's Witnesses' still expect the highest standard of modern medical care and accept most medical treatments (see below). They take full legal responsibility for any adverse consequences directly arising from their decision to decline certain blood products.

5.4 Most adult Jehovah's Witnesses will carry an 'Advanced Decision to Refuse Specified Medical Treatment.' If available, this can be emailed to rduh.ivnurses@nhs.net (Northern Services) or rduh.htt@nhs.net (Eastern Services) for uploading to the patient's EPIC record.

5.5 The Hospital Liaison Committee (HLC) for Jehovah's Witnesses is an important resource for both clinicians and patients. Members of the Hospital Liaison Committee are Witnesses trained to facilitate communication between medical staff and witness patients. They are available to assist at any time at the request of either the treating team or the patient and to facilitate discussion if difficulties arise. Consent for clinicians to discuss patients with the Hospital Liaison Committee is signed within the Advanced Decision. They can be contacted ensuring compliance with RDUH Confidentiality policy. See Appendix 1.

5.6 **Jehovah's Witness views on acceptable and unacceptable medical treatment**

5.6.1 It should be recognised that the guidance offered within this document is equally applicable to other groups or patients (other than Jehovah's Witnesses) and should be considered in the management of other patient groups or individual patients.

5.6.2 **Acceptable Medical Treatments**
Jehovah's Witnesses accept most medical treatments, surgical and anaesthetic procedures, devices and techniques, as well as haemostatic and therapeutic agents that do not contain blood:

- Non-blood volume expanders such as crystalloids (e.g. 0.9% Sodium Chloride, Dextrose) and colloids
- Techniques such as Hypotensive Anaesthesia, meticulous haemostasis and diathermy
- Agents such as Tranexamic Acid, Erythropoietin (except NeoRecormon which contains a trace of albumin and so may not be acceptable to all JW), Aprotinin, Desmopressin, Vasoconstrictors and Recombinant Factor VIIa

5.6.3 **Matters of patient choice**
Each Jehovah's Witness will decide whether he or she wishes to accept the following as a matter of personal choice. It is essential therefore to discuss the acceptability of the following procedures with each individual patient:

- Autologous procedures including intra and post-operative cell salvage, haemodialysis and normovolaemic haemodilution.
- Fractions' of plasma or cellular components eg immunoglobulins including anti-D, clotting factors, cryoprecipitate, fibrinogen, prothrombin complexes, albumin, vaccines.
- RhFVIIa produced by recombinant DNA technology (contains a trace of plasma-derived material)
- Recombinant Clotting Factors

- Blood patches and platelet gels if these are generated and used intraoperatively and using either the patient's own blood or platelets or blood fractions

5.6.4 Unacceptable Medical Treatments

The following treatments are usually unacceptable to Jehovah's Witnesses:

- Transfusions of whole blood, packed red cells, white cells, plasma (FFP) and platelets

- Preoperative autologous blood collection and storage for later reinfusion

5.7 Emergency Admission with Life-Threatening Bleeding in an Unconscious Adult Jehovah's Witness Patient

- Avoid delay and ensure senior assistance. The duty consultant anaesthetist and haematologist must be notified. See Appendix 2.

- Intravenous crystalloids should be used for resuscitation and artificial plasma expanders, which interfere with coagulation, avoided.

- A doctor (Consultant/Associate Specialist), if time permits, should discuss with the patient's relatives the implications of withholding blood and obtain any documentary evidence, for example an Advanced Decision stating that the patient will not accept a blood transfusion even in the event of life-threatening bleeding. A family member or the GP will often have a copy if one exists. This should be placed in the patient's notes and its contents respected.

- If an Advanced Decision is not immediately available but may become obtainable, via the General Practitioner, the family or the Jehovah's Witness Hospital Liaison Committee, avoid blood component or product support if possible until the Advanced Decision has been received, but use clinical judgement in life threatening situations in the interim.

- If serious questions arise regarding the mental capacity of a Jehovah's Witness patient to make decisions about their care, in the absence of a clearly applicable Advance Decision, then the consultant in charge should act in the 'best interests' of the patient, which may involve giving blood or blood derived products. 'Best interests' assessments and the subsequent decisions should be clearly documented in the patient's healthcare record.

- If time permits, a decision to act in this way should be agreed by a second consultant.

- Wherever possible, the family should be kept informed of the steps being taken and these must be clearly documented within the patient's electronic record.

- Under UK law a competent adult is the only person able to consent to or decline treatment. Consent cannot be given by a family member or friend unless prior legal accountability has been arranged with a Lasting Power of Attorney for Health and Welfare. Please refer to the Trust's Mental Capacity Act Policy.

5.8 Children of Jehovah's Witness

5.8.1 General Principles

Jehovah's Witnesses' children will present the same need for clinical decision making as seen when treating adult Jehovah's Witnesses, with the additional issues relating to the child's competency for consent and parental consent.

As with any patient, before examining, treating or caring for a child, you must seek consent (RDUH Consent Policy).

Young people aged 16 and 17 are presumed to have the competence to give consent for themselves. However, as with a competent child, their declining to consent can be overridden by a person with parental responsibility or a court. See Appendix 3.

5.8.2 Children under 16 and Gillick Competence

Children who have sufficient understanding which enables them to fully comprehend the implications of the intervention also have the capacity to consent to that intervention. Gillick competence helps reflect a child's increasing maturity, however it is considered good practice to involve the child's family where possible.

If a child who is Gillick competent or a young person 16-17 years old declines treatment which could lead to death or permanent severe injury, it is possible that the patient's declination to consent could be over-ruled by a person with parental responsibility or by a court.

5.8.3 Children lacking capacity

If a child lacks capacity, consent may be obtained from a person with parental responsibility. Those giving consent must have the capacity to do so, be acting voluntarily and be appropriately informed. Even if the child lacks capacity it is good practice to involve the patient. The welfare principle applies in that the child's welfare or "best interests" are paramount.

5.8.4 Parental Refusal for Blood Transfusion

- BMA advice (Medical EthicsToday, 2004, p141): "it is essential that doctors make every effort to accommodate beliefs rather than resorting to the most obvious medical option that is contrary to the parent's wish or looking to the Courts as a first option."

- If a person with parental responsibility declines permission for a blood transfusion, and despite assistance from the HLC no suitable alternative treatment has been found, then a Special Issue Order must be applied for by the Trust's Legal Department via switchboard.

- Two doctors of consultant status should make an unambiguous, clear and signed entry in the clinical record that the use of blood components or products is essential, or likely to become so, to save life or prevent serious permanent harm.

- A statement will be prepared by the Consultant and faxed to the court. It needs to be proven that the child's need for treatment is so overwhelming for his/her welfare that the parents' wishes must be overridden.

- The parents (and child if appropriate) must be kept fully informed. The doctor only needs the consent of one person with parental responsibility and both parents do not have to agree. However parents should be offered as much support and information as required to support the decision making process recognising clinical factors that may impact on the condition of the child.

- For further discussion and guidance on consent issues surrounding children see Consent: Supported Decision-Making – A Guide to Good Practice (RCS 2016).

- For children who need/are likely to need transfusion of blood components or products repeatedly, for example, in the treatment of cancer or haemoglobinopathy, a Court Order should be sought for the duration of treatment, by the same mechanism. If the parents are seen to obstruct this in a way that puts the child's wellbeing in danger, referral to Multi-Agency Safeguarding Hub (MASH) should be undertaken and the case discussed with the Trust Child Protection Team for how to contact MASH. (RDUH Safeguarding Children Policy).

- If the adult with parental responsibility for a child under the age of 18 declines to allow blood component transfusion and declines to sign the consent form, then the consultant surgeon or anaesthetist may offer to hand write an addendum to the consent form which may enable the parents to sign the consent without recourse to the Courts.

  I / We have discussed with …… (name of consultant anaesthetist or surgeon) the possible need for blood component or product transfusion during this operation / procedure. They have assured us that they will only use transfusion if the clinical situation is life threatening. Having in mind this assurance, but recognising the reality of the doctor's duty and what the law directs, we accept that in a life-threatening situation the treating team will take whatever measures they deem necessary to preserve the life of the child. See Appendix 4

5.8.5 Emergency or life-threatening situations with a child

- If, in exceptional and imminently life-threatening circumstances, it is felt that a delay in treatment with blood components or products could be fatal, a decision to proceed with treatment, against the wishes of parents or guardians and in the 'best interest' of the child should be made.

- This decision must be made by two practitioners of Consultant or Registrar status who are fully informed of the situation, appropriately aware of alternative forms of treatment and agree, whilst treatment is essential, no other treatment options are available.

- The decision must be fully documented in the patient's health record including the names of those with whom it was discussed. See Appendix 5.

6. ELECTIVE SURGERY IN AN ADULT WHO DECLINES A BLOOD TRANSFUSION

6.1 Elective surgery that is associated with a risk of significant blood loss requires advance planning. **See Appendix 6**

6.2 In the surgical outpatient clinic, the issue of receipt of blood products should be discussed. The risks of the surgery should be outlined and the methods undertaken to minimize blood loss described. The patient should have the right to make an informed decision with regards to the risks and benefits of the proposed procedure.

6.3 In the pre-op assessment clinic, the patient should see an anaesthetist to discuss and document which therapeutic agents would be accepted by the patient. Routine blood tests should be requested including: Full blood count, urea and electrolytes, liver function tests, ferritin, transferrin saturation and c reactive protein, vitamin B12 and folate and a coagulation screen. Advice should be given to the patient regarding the discontinuation of any drug associated with increased post-operative bleeding (e.g. warfarin, aspirin). Pre-operative bloods will be reviewed and appropriate pre-operative support, (e.g. Hb optimization) arranged, including any further meetings as required. Patients may be referred to the Haematology team if necessary.

6.4 If a patient has made an Advance Decision to Decline Treatment, this must be uploaded to EPIC. Consent for surgery should be formally documented.

7. OBSTETRIC PATIENTS WHO DECLINE A BLOOD TRANSFUSION

7.1 Patients who are likely to decline a blood transfusion should be identified at the antenatal booking clinic and their views on acceptable and unacceptable treatments clearly documented within their plan of care and plan for management should haemorrhage occur. Such patients should be under the care of a consultant obstetrician

7.2 An Advance Decision, if available, can be emailed to rduh.ivnurses@nhs.net (Northern Services) or rduh.htt@nhs.net (Eastern Services) for uploading to the patient's EPIC record.

7.3 Hb should be checked regularly throughout pregnancy. Iron should be given to maximize stores. Consider intravenous iron infusion in the ante-natal period if oral iron not effective.

7.4 An ultrasound should be performed to identify the placental site.

7.5 If any significant complications are noted during the antenatal period, the Consultant Obstetrician must be informed.

7.6 If unusual bleeding occurs at any time during the pregnancy, labour or the puerperium, the Consultant Obstetrician must be informed and standard management commenced promptly with a lower threshold for intervention.

7.7 The Consultant Obstetrician on-call MUST be contacted as soon as labour is diagnosed. Experienced staff should manage the labour routinely. Oxytocics for the third stage should be actively encouraged.

7.8 The woman must be closely observed in the delivery suite during the postpartum period for at least one hour.

7.9 If a Caesarean Section is necessary, it MUST be discussed with the on-call Anaesthetist and on-call Consultant Obstetrician and carried out by the most experienced surgeon available, paying meticulous attention to haemostasis. Intra-operative cell salvage should be requested and organised for elective sections.

7.10 An intravenous infusion of oxytocin should be considered after the delivery of the baby if the patient has any of the following risk factors:

- Previous history of bleeding or post or ante-partum haemorrhage
- Age (>40 years) and/or maternal obesity
- Multiple pregnancy and /or >4 children
- Difficult operative delivery
- Abnormal placentation / retained products
- Baby weight greater than 3.5kg
- Polyhydramnios
- Fibroids

7.11 When the woman is discharged, she should be advised to report immediately if she has any concerns about bleeding in the puerperium.

8. TREATMENT OF HAEMATOLOGICAL MALIGNANCIES

8.1 It is increasingly possible to treat haematological malignancies without blood component or product support. This will require the MDT to design a patient specific care plan.

8.2 Causes for anaemia should be diagnosed and corrective management commenced promptly. Special consideration should be given to the use of erythropoietic stimulating agents (e.g. EPO) and intravenous iron.

8.3 Prompt consideration should be given to the use of intensive therapies such as high dose chemotherapy and stem cell transplantation (if acceptable to the patient) before the patient becomes excessively debilitated or anaemic.

8.4 Consider the early use of novel therapies with a reduced myelosuppressive effect

9. ARCHIVING ARRANGEMENTS

9.1 The original of this policy/ strategy will remain with the author. An electronic copy will be maintained on the Trust Intranet Hub and Q-Pulse. Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely.

10. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY

10.2 To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
A clear record of the patient's management plan when a blood transfusion is declined.	EPIC	Hospital Transfusion Team

11. REFERENCES

- BMA advice (Medical EthicsToday, 2004)
- Caring for patients who refuse blood. A guide to good practice. Royal College of Surgeons 2016 https://www.rcseng.ac.uk/-/media/files/rcs/library-and-publications/non-journal-publications/caring-for-patients-who-refuse-blood--a-guide-to-good-practice.pdf
- Gillick v West Norfolk and Wisbech Area Health Authority [1985] 3 All ER 402 (HL)

APPENDIX 1 Jehovah's Witness Hospital Liaison Committee (HLC)

The patient should always be asked if they want to contact and have the assistance of the Jehovah's Witness Hospital Liaison Committee.

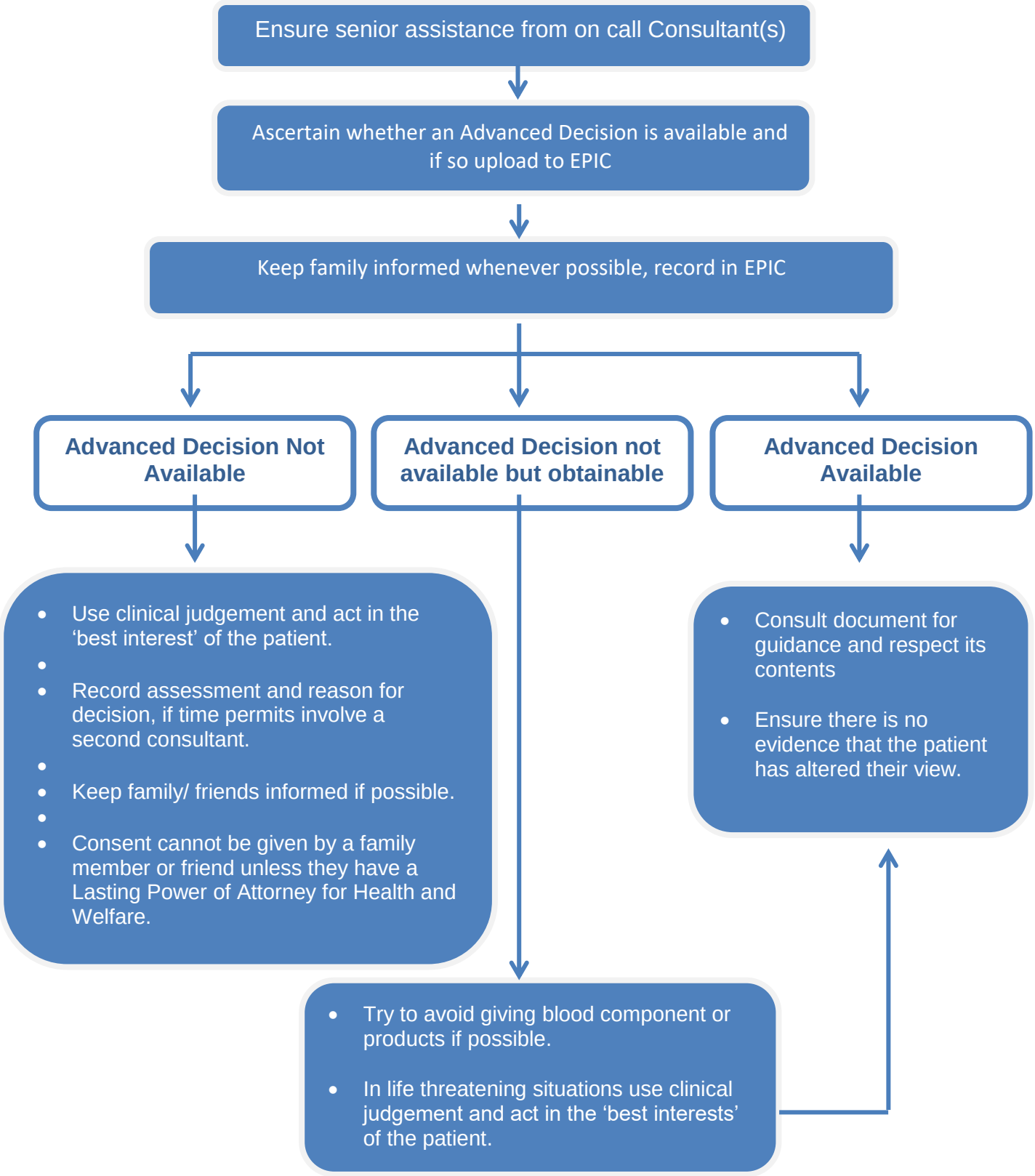
Contacts for Northern Services:

Name	Telephone	Email
Ashley Isaacs	07832 116000	ashleyisaacs@exeterhlc.org

Contacts for Eastern services:

Name	Telephone	Email
Ashley Smith	07789 264532	ashleysmith@exeterhlc.org
James Graham	07894 236537	jamesgraham@exeterhlc.org
James Watkins	07971 069352	jameswatkins@exeterhlc.org
Will Savage	07804 596235	willsavage@exeterhlc.org

APPENDIX 2 Adult Patients who decline blood component or product support who are unable to consent and require emergency treatment



APPENDIX 3 Consent for elective treatment in children where blood component or product transfusion is declined but may be required

Is the child aged 16 or 17 (or younger than this but Gillick Competent)?

YES

- Child consents to blood component or product; proceed as usual.
- Child declines blood component or product support: Adult with parental responsibility can overrule this decision and sign the consent form.

NO

- Consent should be sought from an adult with parental responsibility using the usual consent form for the operation or procedure

If adult with parental responsibility declines blood component or product transfusion or sign the usual consent form, the consultant may offer to write an addendum to the consent form:
I / We have discussed with …… (name of consultant) the possible need for blood component or product transfusion during this operation/procedure/treatment. They have assured us that they will only use blood components or product transfusion if the clinical situation is life threatening. Having in mind this assurance, but recognising the reality of the doctor's duty and what the law directs, we accept that in a life-threatening situation the treating team will take whatever measures they deem necessary to preserve the life of the child. (see Appendix 4 for copy of sentence)

Consider local Hospital Liaison Committee involvement. Contact via Switchboard

<u>If this is still unacceptable</u> to the adult with parental responsibility then a **Special Issue Order** must be applied for by the Trust's Legal Department via switchboard.

Two doctors of consultant status should make an unambiguous, clear and signed entry in the clinical record that the transfusion is essential, or likely to become so, to save life or prevent serious permanent harm.
A statement will need to be prepared by the Consultant and faxed to the court.

APPENDIX 4 COPY OF ADDENDUM TO CONSENT FORM FOR PAEDIATRIC PATIENTS

If the adult with parental responsibility for a child under the age of 18 declines to allow blood component or product transfusion and therefore declines to sign the usual consent form, then the consultant surgeon or anaesthetist may offer to hand write an addendum to the consent form to this effect:

I / We have discussed with …… (name of consultant anaesthetist or surgeon) the possible need for blood component or product transfusion during this operation / procedure. They have assured us that they will only use blood component or product transfusion if the clinical situation is life threatening. Having in mind this assurance, but recognising the reality of the doctor's duty and what the law directs, we accept that in a life-threatening situation the treating team will take whatever measures they deem necessary to preserve the life of the child.

APPENDIX 5 Emergency or life-threatening situation where child or person with parental responsibility decline blood component or product support

A decision to proceed with treatment against the wishes of the child, parents or guardians should be made "in the best interests of the child".

↓

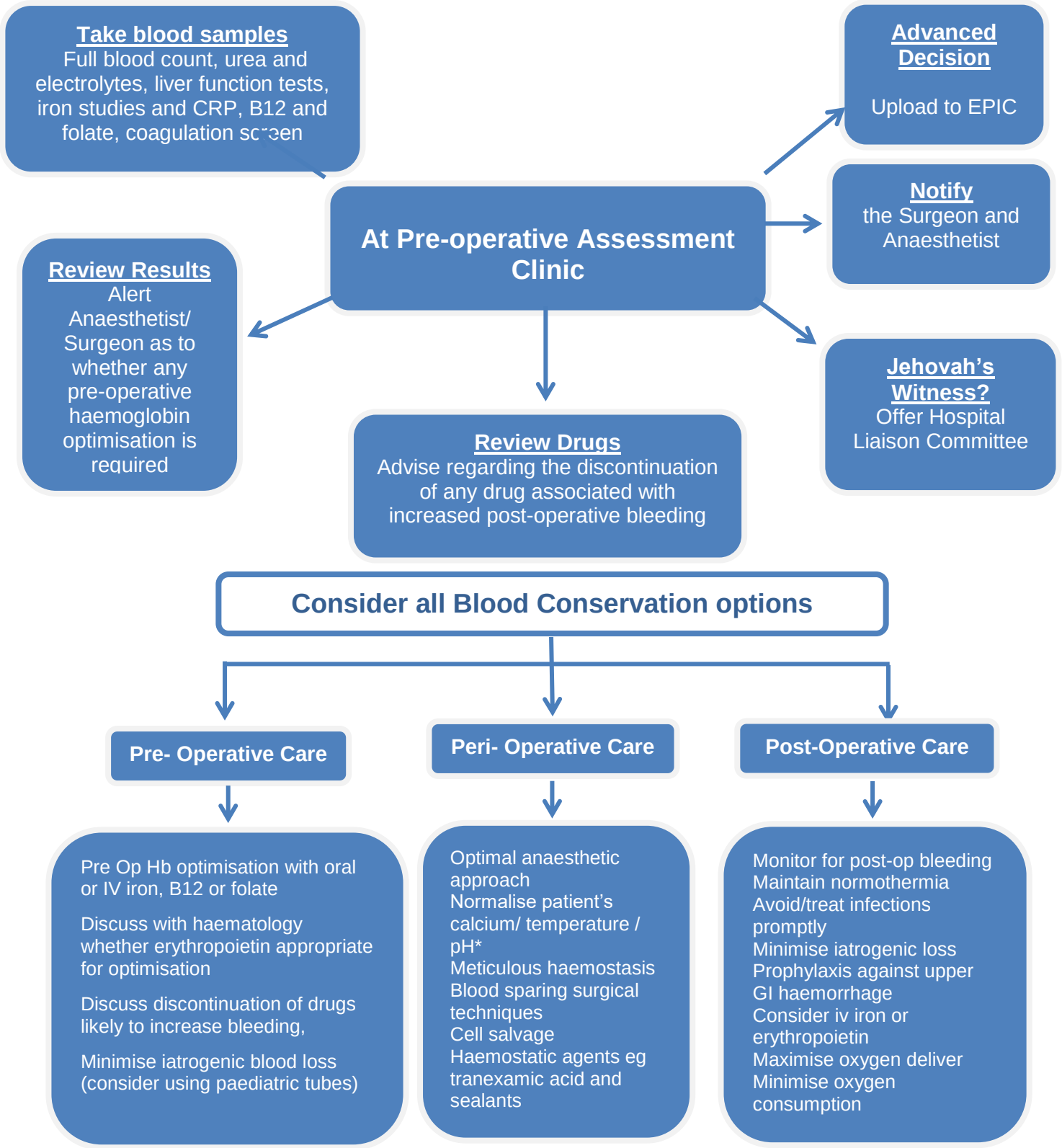
This decision should be made by two practitioners of Consultant status who are fully informed of the situation and who agree that blood or blood component transfusion is essential.

↓

This decision must be fully documented in the notes.

The parents should be kept informed as closely as is possible in this emergency situation.

APPENDIX 6 Adult Surgical Patients who decline blood component or product support



* Platelet function is less effective in patients with hypocalcaemia, hypothermia or acidosis

APPENDIX 7: COMMUNICATION PLAN

COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the policy	All staff involved in the authorisation and administration of blood component and blood products.
The key changes if a revised policy	Updated Trust name and aligned the Eastern Standard Operating Procedure with the Northern Policy
The key objectives	• To provide guidance to staff involved in the care of patients who may decline blood product support. • To provide assurance to patients who decline blood product support that their wishes will be respected.
How new staff will be made aware of the policy and manager action	Cascade by email to Clinical Leads, Ward Managers and Transfusion Champions for dissemination within their teams.
Specific Issues to be raised with staff	Staff should be made aware of the policy.
Training available to staff	Support available from the Hospital Transfusion Team and the Jehovah's Witness Hospital Liaison Committee
Any other requirements	None identified
Issues following Equality Impact Assessment (if any)	Positive impact for Religion / belief ensures that all patients with cultural, spiritual or personal beliefs which impact on their willingness to receive blood products are supported and can make their own choices.
Location of hard / electronic copy of the document etc.	This policy will be stored on the laboratory Q-Pulse document control system and accessible on the Trust Intranet

APPENDIX 8: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	Patients Declining a Blood Transfusion Policy
Division/Directorate and service area	Clinical Support and Specialist Services
Name, job title and contact details of person completing the assessment	Kathleen Wedgeworth Clinical Nurse Specialist Intravascular Fluid Management kathleenwedgeworth@nhs.net
Date completed:	05.12.2023

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

To provide guidance to staff involved in the care of patients who may decline blood product support.

To provide assurance to patients who decline blood product support that their wishes will be respected.

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☐ Staff ☒ Patients ☒ Other (please specify)

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	☐	☒
Disability	☐	☒
Sex - *including: Transgender, and Pregnancy / Maternity*	☐	☒

Race	☐	☒
Religion / belief	☒	☐
Sexual orientation – *including: Marriage / Civil Partnership*	☐	☒

4. **Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to…** (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

Please specify any groups you think may be affected in any significant way

5. **Do you think the document meets our human rights obligations? yes** ☒

6. **Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?**

NA

7. **If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact** please note this below and how this will be monitored/addressed.

"Protected characteristic":	
Issue:	
How is this going to be monitored/ addressed in the future:	
Group that will be responsible for ensuring this carried out:	